PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Landscaping the evidence of intimate partner violence and postpartum depression: A systematic review
AUTHORS	Ankerstjerne, Lea; Laizer, Sweetness; Andreasen, Karen; Normann, Anne Katrine; Wu, Chunsen; Linde, Ditte; Rasch, Vibeke

VERSION 1 – REVIEW

REVIEWER	Necho, Mogesie Wollo University
REVIEW RETURNED	31-May-2021

GENERAL COMMENTS	Comments: General comments: overall the manuscript if thoroughly revised and edited, it might add a great contribution to the scientific community. 1. Authors stated that they systematically investigated articles in PubMed, Embase, Global Health Library, Scopus, and Google scholar were searched for published studies without restrictions on language, time, or study design (up to May 2020). I have two queries from the above description A: an article which was published in October 2020; Necho, M., Belete, A. & Zenebe, Y. The association of intimate partner violence with postpartum depression in women during their first month period of giving delivery in health centers at Dessie town, 2019. Ann Gen Psychiatry 19, 59 (2020) was not included in your systematic review. So I recommend the authors to reconsider this.??? B: the statement….. without restrictions on language…is not clear for me as I cannot understand how you manage articles with different languages??? 2. In the introduction you should discuss articles such as: A Systematic Review and Meta-analysis of Depression in Postpartum Women in a Low-income Country; Ethiopia, 2020: Mogesie Necho1, *, Merid Abadisharew2, Yemyamrew Getachew. 3. The discussion section is too short.. not more than a paragraph, however; the limitation section is more than a page. Do you authors think that this sounds logical? Try to expand ideas in the discussion section before rushing to the limitations.

REVIEWER	Khan, Abdullah Nurus Salam University of South Carolina, Health promotion, education and behavior
REVIEW RETURNED	16-Jun-2021

GENERAL COMMENTS	The work is very interesting. However, I have the following comments: Abstract: Line 38: The acronym IPV in the abstract does not make any sense

	to the readers Article summary: The strengths and limitations are difficult to distinguish. For example, the third and fourth points, are they limitations? Main paper: Line 105-106: I understand what the authors are trying to say, however, “, there no transparency in factors to be adjusted for” remains quite unclear to me. Are the authors saying that the factors affecting IPV and PPD are not well examined? Whichever the authors wanted to discuss should be explained clearly. Also, authors are claiming that case-control and cross-sectional studies are not enough because they are prone to recall bias. It seems like the authors are proposing a better study design in their manuscript which address the methodological limitation. This is not the case because they are conducting a systematic review. Also, the paper itself shows there are more cohort studies selected than the cross-sectional studies (n=20 vs 13). Therefore, this argument seems irrelevant or need a better way of presentation. Line 122-123: Without language? Rephrase…without any restrictions on language, study design, or time. Line 206-207: If the authors do not conduct a pooled data analysis the total number of women is irrelevant information. Line 278-279: The number of studies contradicts what has been mentioned in the result – “Finally, 33 studies – 13 were cross-sectional and 20 cohort studies” Line 304: What is thermology? Line 325-330: Repetitive information, already stated in the background Figure 1: I do not understand why some boxes have the same number of studies as the preceding one in a flow diagram. Besides, lots of grammatical errors are in the boxes as well as in the captions. For example, in table 3, “ Confounders adjusted for in the studies (n=33) categorised of the within the following domains.” I do not know what this means. Figure 2: Poorly constructed graphs, the numbers are overlapping with the bars, full texts are not visible, also what the number means is not explained. Figure 3 and 4, what is the rationale for having separate graphs for HMIC and LMIC

VERSION 1 – AUTHOR RESPONSE

Reviewer 1, Dr. Mogesie Necho, Wollo University			
1.	Authors stated that they systematically investigated articles in PubMed, Embase, Global Health Library, Scopus, and Google scholar were searched for published studies without restrictions on language, time, or study design (up to May 2020). I have two queries from the above description A: an article which was published in October 2020; Necho, M., Belete, A. & Zenebe, Y. The association of intimate partner violence with postpartum depression in	A: Thank you for this suggestion. The article is good and relevant. We have discussed the options for inclusion of the study in our systematic review and decided to stick to the systematic approach throughout the review, without hand-picking	

	women during their first month period of giving delivery in health centers at Dessie town, 2019. Ann Gen Psychiatry 19, 59 (2020) was not included in your systematic review. So I recommend the authors to reconsider this.??? B: the statement….. without restrictions on language…is not clear for me as I cannot understand how you manage articles with different languages???	articles done after the end of the literature search. We have included some of the results from the study in the discussion. B: Thank you for your question on this point. The initial search was without restriction on language. Subsequently, if one of the articles were written in a language which the first author was unfamiliar with it was excluded. In all, 8 citations were excluded for this reason.	
2.	In the introduction you should discuss articles such as: A Systematic Review and Meta-analysis of Depression in Postpartum Women in a Low-income Country; Ethiopia, 2020: Mogesie Necho1, *, Merid Abadisharew2, Yemyamrew Getache w.	Thank you for informing us about this relevant systematic review. We have included the review in the discussion.	
3.	The discussion section is too short.. not more than a paragraph, however; the limitation section is more than a page. Do you authors think that this sounds logical? Try to expand ideas in the discussion section before rushing to the limitations.	Thank you for your comment on this part. We have expanded the discussion to make our statements more nuanced.	
Reviewer 2			
1	The acronym IPV in the abstract does not make any sense to the readers	Thank you for pointing this out. We have corrected the error.	Line 38
2	The strengths and limitations are difficult to distinguish. For example, the third and fourth points, are they limitations?	Thank you for pointing this out. We have tried to rephrase the third and fourth sentences. In addition, we decided to delete the first sentence, because it was more like an objective than a limitation or strength.	
3	I understand what the authors are trying to say, however, “, there no transparency in factors to be adjusted for” remains quite unclear to me. Are the authors saying that the factors affecting IPV and PPD are not well examined? Whichever the authors wanted to discuss should be explained clearly. Also, authors are claiming that case-control and cross-sectional studies are not enough because they are prone to recall bias. It seems like the authors are proposing a better study design in their manuscript which address the methodological limitation. This is not the case because they are conducting a systematic	Thank you for your comment on this. We decided to delete the sentence.	Line 105-106:

	review. Also, the paper itself shows there are more cohort studies selected than the cross-sectional studies (n=20 vs 13). Therefore, this argument seems irrelevant or need a better way of presentation.		
4	Without language? Rephrase…without any restrictions on language, study design, or time.	Thank you for pointing this out. We have changed the phrase.	Line 122-123
5	If the authors do not conduct a pooled data analysis the total number of women is irrelevant information.	Thank you for your comment on this information. Though, our review is not conducting a pooled data analysis we find the total number of women relevant because it gives the reader an idea of how much data the results and conclusion are based on. We have decided to leave this sentence, as in the original manuscript.	Line 206-207
6	The number of studies contradicts what has been mentioned in the result – "Finally, 33 studies – 13 were cross-sectional and 20 cohort studies"	Thank you for pointing out this error. We have corrected it.	Line 278-279
7	What is thermology?	Thank you. Wrong word. We corrected it to terminology.	Line 304
8	Repetitive information, already stated in the background	Thank you for this relevant observation. We have deleted this part.	Line 325-330
9	I do not understand why some boxes have the same number of studies as the preceding one in a flow diagram. Besides, lots of grammatical errors are in the boxes as well as in the captions. For example, in table 3, " Confounders adjusted for in the studies (n=33) categorised of the within the following domains." I do not know what this means.	Thank for pointing this out. Figure 1 has been corrected. Tables 1 and 2 have been closely read, in order to correct errors. Table 3 has undergone a modification to improve the readability. Th e confounders are lined up in bullets and the headline is corrected.	Figure 1. Tables 1 , 2 and 3.
10	Poorly constructed graphs, the numbers are overlapping with the bars, full texts are not visible, also what the number means is not explained.	Thank you for your comment. We have improved the figure.	Figure. 2
11	what is the rationale for having separate graphs for HMIC and LMIC	Thank you for your question. The authors decided to do so because of the great cultural and economic differences that exist between HMIC and LMIC countries, in an attempt to make the results more homogeneous. So, since the result is divided into HMIC and	Figures 3 and 4

		LMIC we decided to do it along with the graph.	